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Delays in awards and funding calls worry NIH-funded researchers

Many programs may be pushed into the next fiscal year, and some could face funding gaps

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The National Institutes of Health faces new funding disruptions. ALISA MACHALEK/NATIONAL INSTITUTES OF HEALTH

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The U.S. National Institutes of Health (NIH) has delayed posting calls for new grant applications for so long that large academic research programs may not have their funding renewed until next year—assuming those notices are approved at all. The slowdown comes on top of concerns that the White House is blocking NIH from spending its congressionally approved budget for the current fiscal year, which ends on 30 September.

Taken together, the biomedical research community is facing the prospect of major funding disruptions again this year, even though last year's chaos has died down and Congress last month rejected President Donald Trump's request to slash NIH's $47 billion budget.

“Many programs are in limbo and may be pushed back a year or two. For some that will be devastating,” says Jennifer Troyer, former director of the division of extramural operations at NIH’s genome institute, who left the agency in December 2025. As for the budget delays, “our concern is that it could drag on for weeks if not months,” says a staffer with a research advocacy group who asked not to be named.

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NIH’s 24 grantmaking institutes and centers regularly publish announcements inviting researchers to submit applications in priority research areas. Early in the Trump administration, NIH tightened the requirements for these Notices of Funding Opportunities (NOFOs); each had to be approved by the NIH director’s office and its parent agency, the Department of Health and Human Services, rather than just by each institute’s advisory council. NIH also said it would halve the number of NOFOs.

The reduction has been considerably sharper. Since Trump took office 13 months ago, NIH has posted only 84 NOFOs, down from 787 in the previous year. Many more are in limbo. NIH has 323 opportunities listed as “forecasted.” Typically, a NOFO is published, or opened for applications, within a few weeks of its announcement. But many on the list were announced in 2024 and ’25 and still are not open.

In a statement, the NIH press team blamed the NOFO delays on the record 43-day federal shutdown last fall. The agency says it is also streamlining NOFOs by making them broader, which should reduce “administrative burden” and improve efficiency. According to a source close to the NIH administration, Deputy Director for Extramural Research Jon Lorsch has said more NOFOs should be released soon.

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NIH officials including Lorsch have said many NOFOs were too narrow and burdened reviewers and applicants alike with unnecessary details. Lorsch’s predecessor Michael Lauer, who is now retired, adds that the agency now funds about 40% of projects through NOFOs, compared with 20% 2 decades ago. Lauer supports paring back the number of NOFOs and making them broader, saying NIH’s emphasis on calls for specific projects risks squeezing out creative, investigator-initiated research. “The pendulum has swung too far,” he says.

NIH program officers worry, however, that fewer NOFOs will erode their ability to nudge fields in new directions and fill gaps. And NOFOs are essential for certain programs such as large networks of centers and efforts to develop research tools, several NIH program officers and leaders say.

There’s little prospect of catching up before the fiscal year ends this fall, says Walter Koroshetz, who was recently removed as NIH’s neuroscience institute chief. Notices for new NOFOs “would need to be on the street by now,” he says. Koroshetz says NIH programs for which Congress specified a funding level, such as the Helping to End Addiction Long-term Initiative addressing opioid addiction, may not be able to spend their appropriations. In that case, they would have to give the money back to the U.S. Department of the Treasury or else meet their spending targets by funding investigator-initiated grants that “look like they fit the program,” which could go against lawmakers’ intent.

The $7.3 billion National Cancer Institute (NCI), NIH’s largest institute, is also feeling the effects. At a recent public meeting, Deputy Director Doug Lowy noted the NOFO approval process now requires 16 steps. He joked that it’s now more complex than a 1929 Rube Goldberg design for a can opener.

Dinah Singer, NCI deputy director for scientific strategy and development, added that NCI is trying to “educate” Lorsch’s office about why some programs must be funded with NOFOs. Clinical trial networks, in particular, involve many academic groups and need the coordination provided by a funding notice. “We are now in a back-and-forth with them to really explain our needs in terms of clinical trials, in terms of infrastructure. … It’s a learning process, and we’re working on it,” she said.

For now, NCI is working on giving its clinical trial networks 1-year funding extensions to hold them into the 2027 fiscal year that starts on 1 October.

The delays in the notices come on top of concerns about new spending restrictions that the White House Office of Management and Budget (OMB) has imposed. OMB issued a circular last year saying it can, for 30 days, limit an agency’s spending to salaries and other essential expenses. As a result, even though Trump signed a law approving NIH’s budget on 3

February, NIH can currently only spend its money for these purposes, *Nature* reported last week. NIH must submit and receive approval for an unusually detailed "spend plan" before it can fund research awards.

To fund grants in the meantime, NIH institutes and centers are using money leftover from a temporary spending measure Congress approved for the first 5 months of the year. That money is mostly being used to cover the annual payments for already approved grants, according to an analysis of NIH data by Jeremy Berg, a former institute director and former editor-in-chief of *Science*. As of last week, the agency has made roughly 800 new and competing awards in total, Berg found; that's less than one-third as many awards as usual at this point in the year.

For the research community, the situation is an unwelcome reminder of last year, when canceled peer-review meetings and other actions by Trump officials resulted in major delays in awards. In July, OMB Director Russell Vought tried to block NIH from spending its remaining budget as part of an effort to claw back unspent funds. Vought released the money only after pushback from a group of Republican senators.

This time Republicans on Congress's appropriations committees have not expressed concern about the current block on the NIH budget, advocates say. But they could be waiting until 5 March, 30 days after Trump signed this year's budget—the deadline for OMB to release the next chunk of NIH's funding.

OMB's press office did not respond to a request for comment.

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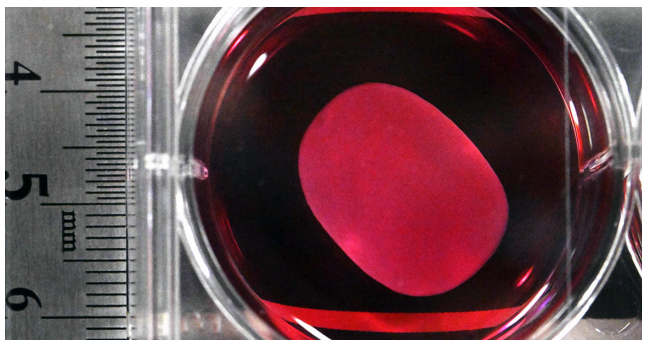

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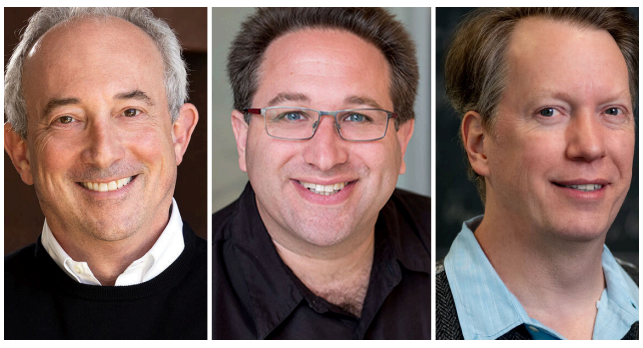

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